

NBIX (Neurocrine Biosciences) — Part 12 workup, first pass

Built 10/6/26 for your review. Nothing is written to the workbook or the score store: these are my proposals for the three judgment indicators (Moat, Revenue Visibility, Management Quality), with the evidence behind each, the composite they give, and the entry-rule checks. Primary sources: Neurocrine's 10-K FY2025, 10-Q and earnings release for Q2 2026 and its 5/18/26 8-K (SEC); Teva's results; Fidelity's research pages (10/6); consensus via yfinance. Press reports are marked as such. The scores and the buy decision are yours.

Where it stands

- Quant side (fresh 10/6): FCF yield 4, revenue growth 5, FCF margin 4 (quant-only Tier 1 about 4.33); valuation 5, ROIC vs WACC 3, relative strength 2, technical 4. No Rule 5 flag; highest correlation with a holding 0.34; P/FCF 23x.
- Proposed judgment scores: Moat 3, Revenue Visibility 4, Management 3 (all medium confidence) → Composite 77.6, BUY. Moat 4 would give 80.6, STRONG BUY; the rubric's cross-checks support 3, not 4 (section 2).
- Rule 12 is a knife-edge: Base needs per-share FCF growth above 10% a year for five years at a flat 23.5x. Consensus is +16% for 2027, but a Medicare price step in 2029 would pull the five-year average to about 9.5% (Base fails); without it, about 11.6%.
- Two live event risks (Rule 11): safety reports on Vykat XR, the \$2.8B Soleno asset, and a Justice Department inquiry into Ingrezza marketing. Q3 results are due 10/26.
- My read: a real candidate, neither a clean pass nor a fail. It needs your judgment on three things (section 7); the 10/22 tranche is too early, 11/7 after the print is the realistic slot.

1. What it is

Product	Q2 2026 sales	Growth / share of sales	Protection and competition
INGREZZA (tardive dyskinesia, Huntington chorea)	\$716M	+15% YoY; 77.8% of H1 product sales; 2026 guide \$2.825–2.875B	22 Orange Book patents expiring 2027–2040; settlements let generics in from 3/1/2038 (10-K). Rival: Teva's Austedo.
CRENESSITY (congenital adrenal hyperplasia)	\$184M	\$53M a year ago; about 15% of diagnosed patients treated	Seven-year orphan exclusivity; patents 2035–2041 (10-K). Competes with steroids; programs in development.
VYKAT XR (Prader-Willi hyperphagia)	\$54M (from 5/18)	\$94M pro forma; 25–30% long-term discontinuation expected	Bought with Soleno for \$2.83B in cash (goodwill \$495M, product rights \$2,238M).

Financials (SEC): TTM to 6/30/26 revenue \$3.37B; FCF after stock comp \$636M (18.8% margin). Cash and securities \$482M, no borrowings, a \$1.0B revolver (4.90%) that was drawn \$600M and repaid in June. H1 2026 operating cash flow was \$282M against \$167M in H1 2025 (the first half is seasonally the weaker one: H2 2025 was \$616M).

2. Competitive Moat Quality: proposed 3 (medium)

Rubric cross-check (a 4 or 5 needs two)	Evidence	Read
(a) Pricing power above CPI without volume loss	Sales +15% on double-digit prescription growth, so price contributed little; management says Medicare pricing stays 'relatively consistent' under multi-year contracting covering 70% of Medicare lives. Medicare negotiation could apply to Ingrezza from 2029.	not evidenced
(b) Gross margin vs industry	Cost of revenues 2.1% of revenue in H1 2026 (about 98% gross margin).	supports the confidence tag only
(c) Retention / switching costs	Chronic daily therapy; 'most preferred and most prescribed VMAT2'; CRENESSITY 'high persistence'. No persistence rates disclosed.	partial
(d) Share trend over 3+ years	Ingrezza's share of the two VMAT2 brands: ~58% (2024: \$2.31B vs Austedo ~\$1.69B), ~53% (2025: \$2.51B vs \$2.26B), ~51% (Q2 2026: \$716M vs \$696M). Austedo grew +34% in 2025 and +40% in Q2; Ingrezza +9% and +15%.	negative

Why 3 and not 4: strictly no cross-check is clean. The real protection is regulatory and legal (patents and settlements to 2038, orphan exclusivity), which the rubric does not list among its four tests, and it is time-limited. Against it: a direct competitor gaining share every year, and Austedo's Medicare price is set under the 2027 negotiation (38% below its 2024 list price, CMS), so its Medicare price falls first. A 4 is defensible only if you weigh the patent runway above the share trend; I would not.

3. Revenue Visibility: proposed 4 (medium)

All three products are chronic therapies, so next-year revenue is mostly the existing patient base repeating, and Medicare contracting covers 70% of lives on stable pricing. But nothing is contractually committed the way a subscription is (compare VEEV 5/M), no persistence or backlog metric is disclosed, and a new product (Vykat XR) is losing 25–30% of patients to discontinuation. That is the same case as ABT's 4/M (a durable repeat-purchase pattern, not literally contracted).

4. Management Quality & Capital Allocation: proposed 3 (medium)

\$M	2021	2022	2023	2024	2025	H1 2026
Cash from operations	256	339	390	595	783	282
Stock-based compensation	134	173	194	196	218	122
Share repurchases	0	0	0	300	168	66
Diluted shares (M)	97.9	98.9	101.0	103.7	102.5	103.9 (Q2)
Acquisitions	0	43	0	0	0	2,830 (Solenio)

Rubric: a CEO new in the role defaults to 3 until a record exists. Kyle Gano became CEO in October 2024 (he was chief business officer before). Credits: the CRENESSITY launch (\$301M in 2025, \$184M in Q2 2026), no debt, buybacks of \$534M since October 2024. Concerns: share count keeps rising despite them (99.7M in 2/2025 to 101.65M in 7/2026) because stock comp outruns repurchases; the first large deal, Soleno, is a single-product, 19%-of-market-cap bet that drew safety reports within three months of closing; a Justice Department civil investigative demand on Ingrezza sales and marketing (8/2025) is open. Leverage trajectory: none before, none now (flat at zero).

5. What the scores give

Moat	Visibility	Management	Composite	Tier 1 avg	Decision
3 (proposed)	4 (proposed)	3 (proposed)	77.6	4.00	BUY
3	4	4	79.2	4.00	BUY
3	5	4	81.2	4.14	STRONG BUY
4	4	3	80.6	4.21	STRONG BUY
2	4	3	74.6	3.79	BUY

Other inputs from the 10/6 quant drafts: valuation 5, ROIC vs WACC 3 (low confidence), relative strength 2, technical 4, analyst consensus 5, volume 2. BUY would mean a 3% initial position (about \$21.6K on \$720K); STRONG BUY 5%.

6. Entry rules and the implied-return test

Rule	Result	Note
5 (momentum floor)	clear	Relative strength 2 (−8.4% vs SPY blended), technical 4.
9–10 (staging)	clear	Above the 200-week MA; monthly MACD positive (5.17 vs signal 5.27): standard initial size.
11 (binary events)	OPEN	Vykat XR: FDA adverse-event database showed 7 deaths and 112 serious cases of 654 total by 7/31, 535 of them this year (press and patient-group reports; causality unknown). Neurocrine stands by the drug; I found no FDA action. Also the DOJ inquiry, and the 10/26 print.
12–13 (Base >10%, Bear ≥0%)	knife-edge	See the table below.
14 (50x ceiling)	clear	P/FCF 23.5x on diluted shares.
15, 16, 18, 38	clear	Healthcare 4.5% → 8.5% with VEEV → 11.5% with NBIX; the 3% position needs about \$12K more SPMO sold to hold cash above 5.5%; correlation 0.34.
Per-share FCF growth		
	Base	Bear (growth halved, terminal ×0.81)
8% a year	+8.0%	−0.3%
10%	+10.0%	+0.7%
12%	+12.0%	+1.6%
2027 +16%, 2028 +12%, 2029 0% (Medicare step), then +10% (9.5%/yr)	+9.5%	+0.4%
2027 +16%, 2028 +12%, then +10% (11.6%/yr)	+11.6%	+1.4%
		Bull (+4 points, terminal ×1.1)
8% a year		+14.2%
10%		+16.2%
12%		+18.2%
2027 +16%, 2028 +12%, 2029 0% (Medicare step), then +10% (9.5%/yr)		+15.7%
2027 +16%, 2028 +12%, then +10% (11.6%/yr)		+17.8%

Basis: price \$143.73, TTM FCF after stock comp \$636M on 103.9M diluted shares = \$6.12 (23.5x); the terminal multiple is today's, because the five-year median is far higher (FCF was near zero in 2022–23); no dividends. Base clears 10% only above 10% growth; Bear stays ≥0% down to about 8.7% growth. Consensus (yfinance): non-GAAP EPS \$10.21 for 2026 and \$11.88 for 2027 (+16%); the long-term growth the street uses is about 12%. The 2029 step is my assumption, not a forecast: Ingrezza's small-biotech exemption from Medicare negotiation lapses after 2028 and Teva's Austedo is repriced in 2027; how much of Ingrezza's Medicare revenue a negotiated price would remove is not disclosed.

7. What I need from you, and the clock

1. Moat: 3 (the rubric as written) or 4 (weighing patent runway over share loss). It decides BUY or STRONG BUY.
2. Rule 12 Base: which growth path is the Base case. Without a 2029 Medicare step it is 11.6% (clears 10%); with one, 9.5% (does not). The exemption lapses by statute, so a Base without the step is the optimistic reading; my lean is to include it, in which case NBIX fails the Base test whatever the price (the method caps the terminal multiple at today's, so a lower price does not help). The alternative is a written case that Crenessity, Vykat XR and the pipeline fully offset it. Before the print I will pull the street's 2028–29 estimates to see what they already assume.
3. Rule 11: treat Vykat XR safety as in the thesis (then I document it and size down), or wait for the FDA's and the company's response and the 10/26 print.
4. Status on CANDIDATES LIST: Needs Initial Screen → Ready for Tier 1 (quant 4 / 5 / 4, no Rule 5 flag). I have not changed it.
5. Clock: Q3 results 10/26 (Fidelity), so the 10/22 tranche is too early; the 11/7 tranche is the first realistic slot. I will re-check the safety reports, any FDA response and the print before then. SPOT (reports 10/22) gets the same workup next if you want a second name.

Sources: SEC filings for Neurocrine (10-K FY2025; 10-Q Q2 2026; 8-K 5/18/26; Q2 2026 earnings release and call transcript via MarketBeat); Teva Q2 2026 and FY2025 results via press summaries; CMS 2027 negotiated prices via Becker's and Manatt summaries; Healio, STAT, Becker's and Fierce Pharma for the Vykat XR reports (secondary, tier 5); Fidelity research pages 10/6; yfinance consensus.